



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ايمان سعد مهدى سلطان
المحافظة : الجيزة
الحى / المدينة : اوسيم
رقم البطاقة : 27508190101121 رقم التليفون : 01144884076
العنوان : الجيزة
العمر (تاريخ الميلاد) : 19-08-1975 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ايمان سعد مهدي سلطان National ID: 27508190101121	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
مملكة البحرين

إيمان
سمت مهدي سلطان
٣ ش حسن الجندى - ش المصرف
مركز أوسيم - الجزيرة

٢٧٥٠٨١٩٠١٠١١٢١

LG0884481

٢٠٦٥/٠٧ ٢٧٥٠٨١٩٠١٠١١٢١

زينة منزل

أنتى
مستلمة
مقرولة
يحي اسماعيل عبد الله اسماعيل
البطقة سارية حتى ٢٠٣٢/٠٧/٠٧

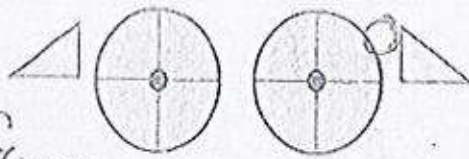
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ST حوله ك ريف

Dms
لساوى
موضح

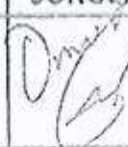
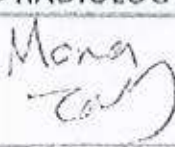
MDT FORM 10/12/2025 G

Name	ليلى محمد	MRN	3089 / 2025
Age	50	ID	
Menopausal status	Pre-menop	Phone	01144884070
NRS SCORE	1.5	Marital status	Married
BMI		Family history	Sister "T20"
CO morbidity	HTN	REASON OF MDT	New Case.

3/12/25 Lt breast lump Clinical Data BX: HOC				R Lt axilla tail (mod) 4cm 
ER:	PR:	HER 2:	Ki67:	
8/8	4/8	+3	25%	
Sonomammography: ACB B Lt breast VOG max 5.5x2x2.5 cm + Non max 2.9x1x2.7 cm. + Multiple axillary LNs 2.5 cm				TNM
Mets Workup: CT CHEST 9/11/2025 Lt axilla 2x1.8 cm. Lt INC 0.8 Lt axilla 1.7x1 cm.				

Recommendations: Rx bilOvaries nodules 3-4 mm likely benign.
 Dr. Echo + Labs @
 Slide Review @
 Targeted US on Rt axilla @
 MDT - NACT TCHP @
 11/12/25

MDT OUTCOME & PLAN
 - NA TCHP X60
 Best (best Rx), pt can be for
 MDT (TCHP X60)
 Re-Monitoring CEST.
 12/12/25

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					



ImmunohistoChemistry report

Report Number: m1617

Female :	النوع	امان سعد مهدي	المريض
القاهرة :	العنوان	50 Y	سن
08/10/2025 12:47 :	تاريخ استلام العينة		تاريخ اخذ العينة
13/10/2025 09:58 :	تاريخ التقرير	مركز صحة المرأة	الب التحليل
		مركز صحة المرأة	ناتجة

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by : - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor (PR) (IE2) Rabbit monoclonal primary antibody.
Sample recieved was treated by : - VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA .
Sample recieved was treated by : - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: strong nuclear staining of 95 % of tumour cells
PR: moderate nuclear staining of 5 % of tumour cells
HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.
Ki67 : positive nuclear staining of about 25 % of tumour cells

Referral Lab :

Comment :

Conclusion :

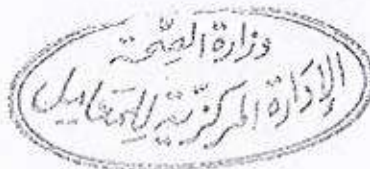
ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 4 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3 +)
#KI67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 25 % .
(CORE BIOPSY)

Pathologist:

د. ايناس رمزي

Lab Manager:

د. نورا ماهر



Patient Name: Eman Saad Mahdy Soltan	Date : 09/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 3089/2025	Pat. ID: 132824

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS
REVEALED: -**

1. Chest:

- Left breast architectural distortion.
- Left axillary level I and II pathologically enlarged lymph nodes the largest measures 2×1.8 cm.
- Prominent left internal mammary lymph node reaching 0.8cm.
- Prominent right axillary lymph nodes , the largest 1.7×1 cm .
- Few bilateral scattered tiny pulmonary nodules ranging from 3-4 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *Left breast architectural distortion with ipsilateral axillary lymph nodes as described.*
- *Bilateral scattered pulmonary nodules likely benign in nature for follow up.*
- *No definite hepatic metastatic deposits .*

Signature reham kheder
12/11/2025 4:23 PM

CT Scan

[[DOCTYPE:CT_SCAN]]



Women's Health Outreach Program

Name: Eman Saad Mohamed

Date: 17-09-2025

Bilateral full field Digital Mammography

Clinical summary: 50 yrs old female patient seen for a recently felt lump in the left breast

Technique

Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibroglandular dense mammary parenchyma (ACR B)
- A subtle area of architectural distortion is noted at the left breast UOQ posterior thirds, with associated suspicious microcalcification confined to the area of distortion. The area measured about 5.5 x 2 x 2.5 cm (AP X T X S).
- No Masses identified
- Ultrasound:
- Homogeneous parenchymal echopattern
- A focal non-mass area of altered parenchyma is noted at the left breast 2-3 o'clock position 7 cm from the nipple measured about 2.9 x 1 x 2.7 cm in maximum dimensions. It showed echogenic calcific foci within consistent with mammography findings. Doppler ultrasound showed peripheral vascularity.
- No solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Multiple prominent axillary lymph nodes showed diffuse cortical thickening of 2.6 mm. except for one small lymph node at level I showed irregular margin and effaced hilum measured about 1 cm and cortical thickening of 3 mm (suspicious), it is identified at the medial aspect of the axilla, deeply seated, at axial plane, over lying axillary vein. Non-specific axillary LNs.

Right breast

- Scattered fibroglandular dense mammary parenchyma (ACR B)
- No, distortion or suspicious clusters of microcalcifications.
- Ultrasound:
- Homogeneous parenchymal echopattern
- No solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Left breast: a palpable calcified area of architectural distortion with one suspicious axillary LN as describe (BI-RADS 4c)

Right breast: Normal study. (BI-RADS1)

Recommendations:

Biopsy and histopathological correlation.

Best regards,
Dr. Iman Ewais, MSc

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Medical ID: 3089/2025	Pat. ID: 132824

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Signature reham kheder
12/11/2025 4:23 PM

Name: Mrs. Eman Saad Mahdey

Date: 2026-01-01

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 50-year-old female with pathologically proven left breast IDC grade II triple positive molecular subtype, re-assessment study

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ, posterior & middle thirds focal asymmetry with overlying suspicious microcalcifications and surrounding distortion.
- **Ultrasound** demonstrated 2-3 o'clock, posterior and middle thirds, 4 mm deep to the skin, 11.4 cm from nipple, ill-defined area of altered parenchyma with posterior shadowing, microcalcifications and mild internal vascularity within, mounting to mass formation measuring about 36 x 19 mm.
- Two level 1 indeterminate axillary LNs are noted showing cortical thickness of 4.5 mm, no pathological infra or supra-clavicular lymph nodes.

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 6): UOQ pathologically proven malignant process with two ipsilateral level 1 indeterminate axillary lymph nodes.

Recommendation:-

For CESM.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



Pat. ID: 3514187	Pat. Name: Email: Saad Maher
Date: 08/03/2026	Procedure Name: Bone Scan

Tc-99m MDP BONE SCAN:**Procedure:**

- Whole body skeletal survey was obtained 3 hours post IV injection of 21 mCi of the tracer.

Findings:

- No abnormal focal active or cold osseous lesions could be detected to account for bone metastases
- Degenerative changes all over the spine are noted
- Osteo-arthritic changes of shoulders , knees and ankles are also observed.

Opinion:

- The skeleton is free from metastases.

Best Regards,

Prof. Dr. Maha Abd Elkareem

Professor of Nuclear Medicine

Kasr al Ainy, Cairo University

ECHO REPORT

NAME: *أحمد بن عبد الله*

DATE: 22 / 2 / 2026

ID NUMBER: ٢٠١٩ / ٢٥

SEX:

AGE:

EF	67%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD	2.8	2-4cm	LA	3.6	2-4cm
FS	30%	25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function Normal

SWMA No. Rum A

LA and Ao Root Dimensions Normal

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve

Right Side and Systolic Function TAPSE = 2.3

Intracardiac Masses and Effusion

DR. *أحمد بن عبد الله*